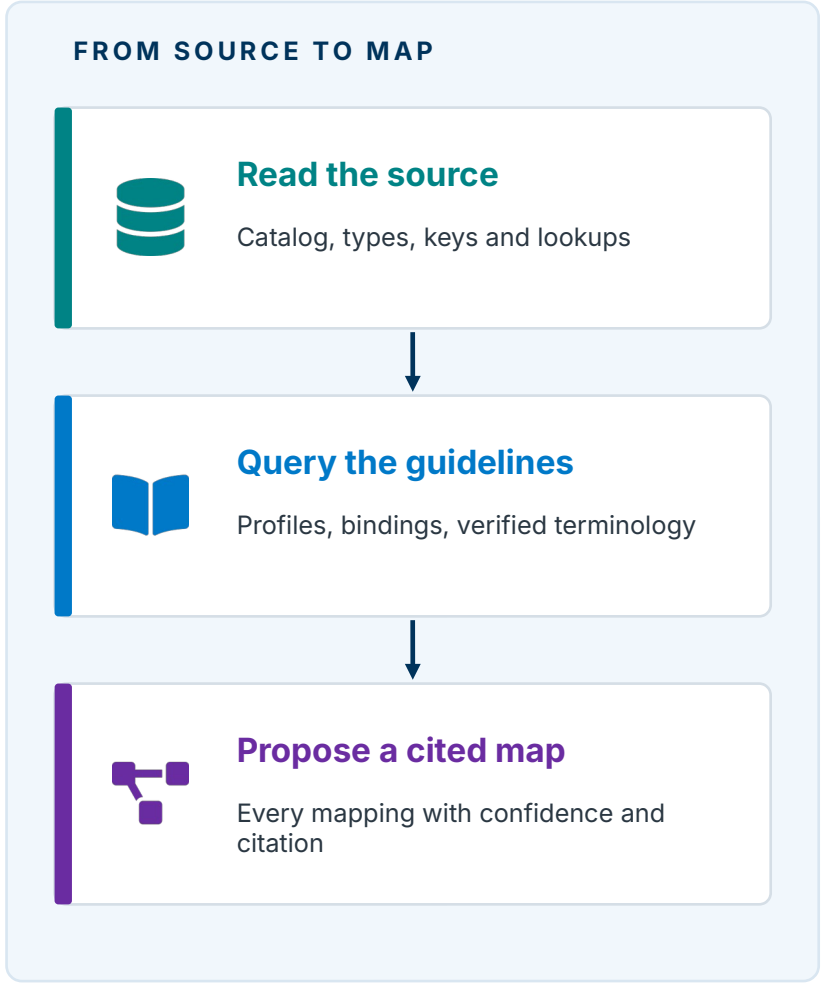
Know your guidelines

Clinical Interoperability Copilot

A tool to optimize the processes of interoperability projects: from a source database to a cited FHIR mapping.



The problem: fragmented clinical data

Healthcare organizations must exchange clinical records using HL7 FHIR to meet regulatory requirements. But clinical data is fragmented across multiple systems, each with different formats, structures and technologies.

Source: fragmented systems

- HIS, LIS, RIS and PACS behind an integration engine
- Multiple unknown data models
- Local codes: HGB, CC001, XRCH2V
- Undocumented columns, implicit keys

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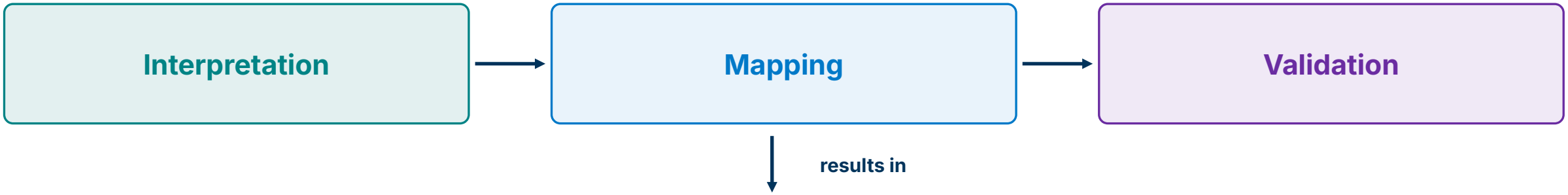
Target: FHIR R4 plus an IG

- Profiles, cardinalities, must-support
- Bindings to SNOMED, LOINC, RxNorm
- References between resources

Mapping one to the other is where interoperability projects spend their time.

The pain: errors, ambiguity and long cycles

Compliant FHIR resources do not come for free. They require complex work, every time.



Errors

Mappings that do not conform.

Ambiguity

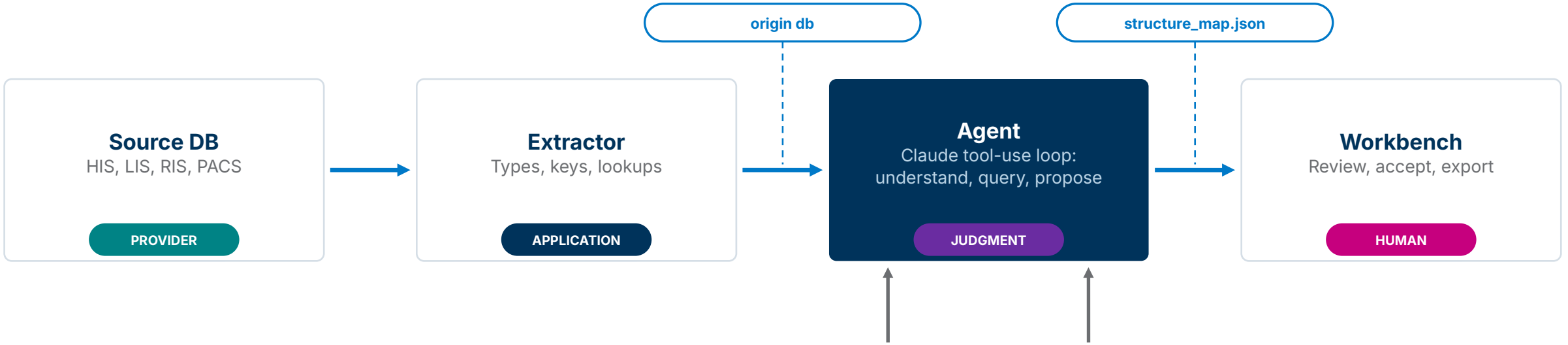
Several valid readings, no clear answer.

Lengthy cycles

Integrations that take months.

The challenge: accelerate FHIR adoption and implement interoperable clinical flows in FHIR.
That means turning fragmented clinical data into consistent, compliant, and interoperable information.

Architecture: facts in, cited map out



Decoupled by design

Two JSON contracts. Run, test or replace either half alone.

Implementation Guide

guide.db with five query tools

Terminology

Verified SNOMED, LOINC, RxNorm, ICD-10

Know your guidelines: the agent never writes a standard code from memory; every mapping is cited.